

METRO CITY HOSPITAL

PATIENT FACE SHEET

Patient Name: Anita Sharma Age / Sex: 67 / Female
MRN: 552901 Blood Group: O+
Address: 14 Lake View Road, Pune Phone: 98xxxxxx12
Date of Admission: 03/05/2026 Date of Discharge: 11/05/2026
Attending: Dr. R. Menon (Internal Medicine)
Consultants: Pulmonology, Nephrology

Known Allergies: NKDA (No Known Drug Allergies).

Emergency Contact: Vikram Sharma (son), 99xxxxxx45.

METRO CITY HOSPITAL - ADMISSION NOTE

Reason for Admission / Date: 03/05/2026

Chief Complaint:

Fever, productive cough and breathlessness for 5 days.

History of Present Illness:

67F, hypertensive, presents with high-grade fever, purulent sputum, pleuritic right-sided chest pain and exertional dyspnoea. No haemoptysis.

Past History: Hypertension (10 yrs), Hypothyroidism. Ex-smoker.

Provisional / Principal Diagnosis:

Community-Acquired Pneumonia (right lower lobe).

Secondary Diagnoses: Hypertension; Hypothyroidism.

Examination: T 39.1C, HR 104, BP 148/88, SpO2 91% on room air, RR 26. Right basal crepitations.

METRO CITY HOSPITAL - PHYSICIAN PROGRESS NOTE

Day 3 (05/05/2026)

Persistent hypoxia despite antibiotics. Sudden desaturation episode.
D-dimer markedly elevated. CT pulmonary angiogram ordered (see imaging).

Revised / Final Diagnosis:

Acute Pulmonary Embolism (segmental, right lower lobe).
Pneumonia now considered less likely given imaging.

Relevant labs today: Serum Potassium 4.2 mmol/L, Creatinine 1.9 mg/dL.

Plan: start anticoagulation (heparin -> warfarin), continue O2.

METRO CITY HOSPITAL - LABORATORY REPORT

Collected: 04/05/2026

Investigations / Biochemistry:

Serum Sodium 137 mmol/L
Serum Potassium 5.8 mmol/L (3.5-5.1) *HIGH*
Serum Creatinine 2.3 mg/dL (0.6-1.1) *HIGH*
Urea 64 mg/dL
CRP 188 mg/L
D-dimer 4.9 ug/mL *HIGH*

Haemogram:

WBC 17,800 /uL
Platelets 388,000 /uL

Microbiology:

Blood Culture (04/05) RESULT AWAITED.
Sputum Culture PENDING.
Pleural Fluid Cytology sample sent, REPORT AWAITED.

METRO CITY HOSPITAL - RADIOLOGY REPORT

Study: CT Pulmonary Angiogram (CTPA) Date: 05/05/2026

Findings:

Filling defect in the right lower lobe segmental pulmonary artery, consistent with acute pulmonary embolism. Small right pleural effusion. Patchy right basal consolidation also noted.

Impression: Acute segmental PE with small pleural effusion.

METRO CITY HOSPITAL - PROCEDURE NOTE

Procedure: Diagnostic Thoracentesis (right) Date: 06/05/2026

Operator: Dr. S. Iyer (Pulmonology)

Indication: right pleural effusion, diagnostic sampling.

Under aseptic precautions and ultrasound guidance, 60 mL of straw-coloured pleural fluid aspirated and sent for cytology and culture.

No immediate complications. Post-procedure CXR: no pneumothorax.

METRO CITY HOSPITAL - MEDICATION ADMINISTRATION RECORD

(Inpatient / on admission)

Medication Name	Dosage	Frequency
Tab. Amlodipine	5mg	1-0-0
Tab. Thyronorm	50mcg	1-0-0 (before food)
Inj. Meropenem	1g	1-1-1 (IV)
Inj. Heparin	5000 IU	SC BD
Tab. Valproate	500mg	1-0-1
Inj. Emeset	4mg	IV SOS

Note: Valproate continued from home (seizure prophylaxis, old CVA).

METRO CITY HOSPITAL - DISCHARGE SUMMARY

Date of Discharge: 11/05/2026

Course in the Hospital:

Treated for acute PE with anticoagulation; AKI improved with hydration. Thoracentesis performed for effusion. Afebrile from day 6.

Discharge Medications (Medication Name / Dosage / Frequency):

Tab. Warfarin 5mg	0-0-1 (start anticoagulation for PE)
Tab. Amlodipine 10mg	1-0-0
Tab. Thyronorm 50mcg	1-0-0
Tab. Valproate 500mg	1-0-1
Tab. Tramadol 50mg	1-0-1 (pleuritic pain)
Tab. Emeset 4mg	1-0-1 SOS

(Meropenem and Heparin stopped at discharge.)

Condition at Discharge: stable, SpO2 96% on room air, ambulant.

Nursing Progress Note 08/05/2026

Pt resting comfortably, afebrile this shift.

O2 weaned to room air, SpO2 95-96%.

*Anticoagulation started - watch for bleeding,
gums / urine. INR sample sent.*

Pt anxious about going home, reassured.

Pain score 3/10 - given Tramadol as charted.

- Staff Nurse, Ward 5B

Doctor's Order Sheet 10/05/2026

1) Warfarin 3 mg PO once daily at night.

*** check INR in 3 days, target 2-3 ***

2) Continue Valproate 500 mg BD.

3) Stop Meropenem - course complete.

4) Amlodipine - increase to 10 mg OD

(BP still 150/90).

5) Tramadol 50 mg BD PRN for pain.

- Dr. R. Menon

Allergy & Vitals Chart 10/05/2026

ALLERGY ALERT: SULFA drugs - rash (per pt's daughter).

Band applied. Charted in red.

Vitals (10/05):

Temp 98.6 F HR 80 BP 150/90

RR 18 SpO2 96% RA

Weight 62 kg. Intake/output adequate.

Discharge Counselling Slip 11/05/2026

Discharge counselling done with pt + son.

Explained warfarin - take same time daily,

avoid NSAIDs, watch for bleeding.

Diet advice given. Pt verbalised understanding.

*(Next-visit / follow-up date NOT yet fixed -
to be decided by OPD.)*

- Staff Nurse, Ward 5B